

Patient Perspectives: Barriers to Complementary and Alternative Medicine Therapies Create Problems for Patients and Survivors

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There are multiple barriers to better understanding of complementary and alternative medicine (CAM) approaches and the use of natural substances to reduce cancer risk. CAM approaches are not patentable, and the pharmaceutical industry has no interest in the area. Research generally approaches concepts from the deconstructionist side: there may be interest in herbal mixtures but its “active” constituents are explored one at a time, missing any synergistic or even additive effects. Little attention has been paid to environmental pollutants and exposures. Most patients believe these are active causes of their disease, but researchers are still exposing animals to one chemical one time, which fails to duplicate human exposures. Attention to lifestyle issues is presented almost solely in terms of obesity—a concept that is a barrier to change by itself. Research animals rarely if ever are fed dietary components that humans normally eat, such as soda or diet soda, potato chips, and high-fructose corn syrup, artificial colors, artificial flavors, other additives, or combinations of bad fats with too much sugar, too much salt, and artificial ingredients. Long-term treatment effects on survivors may not necessarily require a solely pharmaceutical approach. Drugs to treat each condition separately may require even more drugs to offset the unwanted effects of each. Whole systems approaches to research are urgently needed. A final barrier is that once a treatment is accepted in mainstream, it ceases to be considered “alternative.” The genesis is soon forgotten and the remaining alternative modalities continue to be viewed with suspicion, doubt, and misunderstanding, to the detriment of both patients in treatment and cancer survivors.

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I often attend medical, scientific, and research meetings. My attendance is cross-disciplinary, and I only occasionally see the same people at different groups’ meetings. So my perspective is a bit different than most: I think of it as a broad overview. I am an advocate. I do not practice any method of medicine,

whether conventional or alternative. I have devoted the past 14 years of my work to providing information, education, and advocacy for people with cancer, their families, and friends.

The United States has approximately 10 million cancer survivors. Many suffer aftereffects of treatment. Studies and surveys show these people are interested in natural approaches to reducing pain, preventing recurrence, as well as in making lifestyle changes.¹⁻¹⁰ Has the research community succeeded in addressing the concerns of these patients? It is my impression that they have not, and that there are multiple barriers standing in the way of this goal. This article discusses these impressions from the viewpoint of the leading patient advocacy organization concerned with complementary and alternative medicine.

Active Constituents

At some conferences I hear studies highlighting one particular constituent or element of a natural substance. The dedicated researcher explains how their particular substance has the potential to be the most potent, the newest, the best solution for . . . fill in the blanks with your favorite problem or solution. Almost everyone is searching for the “magic bullet.”^{11,12} Everyone protests that they are not, but I listen and I hear them hoping this substance, this element, this patentable constituent, will be “the one.”

I see this type of thinking as a barrier to be overcome—one of many. As so many people who work in the pharmaceutical industry have publicly stated, they are not at all interested in substances that cannot be patented. So research on a whole substance, rather than a single constituent, is not moved very far along. Scientists can “discover” the value of a vitamin or

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group of vitamins, a nutrient or group of nutrients, but there is no motivation, spelled MONEY, to move the research forward.

And that is a gigantic barrier!

The deconstructionist, reductionist approach to research, which many tell me is supported and encouraged, not to say enforced by the way the National Institutes of Health evaluates grants, is another major problem. Some advocates as well as scientists believe that an answer may lie in combining many valued substances. Indeed, additive and synergistic effects of whole foods combined with stress reduction have already been demonstrated in cholesterol-lowering and heart disease prevention programs. Thirty years of data support this approach.^{13,14}

Some researchers have already noted that natural compounds from plants “address whole mechanisms, sets of pathways that interfere with tumors rather than single enzyme inhibition accomplished through use of cytotoxic drugs.”¹⁵

Combinations

At other conferences I hear researchers laud the value of exercise—usually stating that it needs to be done 60 minutes a day or some other unachievable level. This type of finding is a barrier to people using exercise for health because the majority of people are *not* athletes or even athletic. I know, I am one of the “not” athletes. But if we could look at research that talked about combining exercise with nutrition, the benefits might be greater and more easily achievable.¹⁶ But who will do this research? Exercise is almost always discussed in the context of obesity. Talking to people about reducing their weight in a culture that advertises and promotes unhealthy food and behaviors is extremely difficult, easily misunderstood, and generally insulting.

Small steps could move a patient or survivor away from the standard American diet (called SAD for the most obvious of reasons), without accusations or guilt trips. Step 1 starts with adding fruits and vegetables to a nutritional program. Step 2 is removing one unhealthy component (I would suggest starting with soda, which has no nutritional value and is a pure waste of time, energy, calories, and money, supplemented with superb advertising).¹⁷

Awful Human Food, Excellent Animal Diet

I often speculate whether outcomes would be different if our research animals ate like human beings—soda, artificial color, artificial flavor, potato chips, high-fructose corn syrup, artificial sugars, and the like.¹⁸ Would they fare much worse, would there be unexpected toxicities? My favorite clinical trial line of all is “there were

no unexpected toxicities,” which seems to be designed to make us feel better about the terrible unwanted (side) effects of cancer drugs. I generally avoid calling them “side” effects because they *are* the effects and often affect as many people as the desired effects.

Patients in the Middle

Things are better now, but when I was diagnosed in 1993, I felt like I was being pulled in different directions between practitioners who did not speak to each other. On the one side were my acupuncturist (willing to talk to my doctors) and my chiropractor (willing but skeptical), and on the other, my surgeon, medical oncologist, radiation oncologist (unwilling and very skeptical). How can a patient proceed with no real contact among her health care providers? Some cancer centers now offer integrated treatments, including massage, yoga, acupuncture, nutrition, and others. These should be available to all.¹⁹

Some lucky folks have conventional doctors who ask them what else they do, and they can talk openly. Some have physicians who have learned a bit about nutrition or dietary supplements, have met acupuncturists or naturopaths, and thus take down one barrier.²⁰ But studies continue to show that many people using CAM do not discuss it with their health care providers. Patients remain, then, caught squarely in the middle.

Move From Alternative to Mainstream

Another interesting thing to me, and another barrier to understanding complementary and alternative approaches, is that when a technique or therapy becomes accepted into the mainstream, it ceases to be alternative. (For instance, the American Cancer Society removed hyperthermia from its Unproven Cancer Therapies list in 1977, and it was approved for cancer treatment by the FDA in 1984. It nevertheless remains difficult to find in the US, and current NCI material does not acknowledge its alternative origin.²¹ Its genesis is soon forgotten, and the remaining alternative modalities continue to be viewed with suspicion, doubt, and misunderstanding.

Evidence Needed to Prove or Disprove

At a recent AACR meeting, an oncologist told me that he would never allow his patients to use supplements. When I asked his rationale, he said it was contraindicated inasmuch as chemotherapy-created free radicals and antioxidants reduced them. I suggested that perhaps we ought to have evidence to prove that supplements helped cancer cells before making a blanket rejection. He was angered by this suggestion. Another

barrier is thus the lack of information on the single most asked question patients want to know, "Is it okay to use antioxidant supplements with chemotherapy?" Until we have randomized multicenter trials in this area—that is, Level I evidence—patients and oncologists will continue to have a communication gap.

The Patient Track

Research ignores the reality of most people's experiences. Many people with cancer reach an end to treatment, or the treatment does not work for them, or they simply choose to add CAM for a better shot at healthy survival. Research is far behind patients in this. I propose that we develop an entirely new way to do research—one that can be added to what already exists. It could be called the Patient Track, and it would seek outcomes based on what people are already doing.

A recent study demonstrated that 34% of human research subjects in a Phase I trial were using some form of CAM. In 2004, Dy et al showed that 90 out of 102 participants in Phase I trials at the Mayo Clinic were already using CAM.²² They suggested, as I did back in 1998, that CAM use should be ascertained to see if there were "adverse effects and/or efficacy." If these suggestions had been followed, CAM use would have been followed as part of every research study—and by now we would have some interesting data to inform those Level I trials.

Whole Systems Research

And, of course, a barrier to CAM is the way information is measured in clinical trials. The 3-phase clinical trial system was set up to facilitate drug approval. However, it is only when a drug is being used by the "general public" that the real results are known. In CAM research, I question whether we will even be measuring the results that matter to patients if we continue to follow the drug-based, reductionistic approach. Which current studies are truly measuring a holistic, whole-body response? I haven't seen them at conferences I attend. New quality of life measurements need to be developed that are truly relevant to CAM practice. Whole systems research is needed,²³ yet it is barely on the radar screen of the major funding agencies.

Conclusion

We cannot continue to ignore the realities of current CAM research or the integration of CAM and conventional medicine. Although the Annie Appleseed Project advocates for people with cancer using CAM, we are not creating this constituency; it already exists. Until these problems are addressed, vast numbers of cancer patients who use CAM will continue to be served inadequately by the medical community.

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